PC-05 Pediatric Narrow-Complex Tachycardia

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Pediatric narrow-complex tachycardia (QRS at or below 0.09 seconds) with suspected SVT or serious symptoms. Treat sinus tachycardia by correcting its cause.

Field flow

1. Assess	Perfusion, mental status, BP, respiratory status, onset, P waves, R-R variability, and reversible causes.
2. Compromise?	Hypotension, acute altered mental status, shock, or sudden collapse: prepare synchronized cardioversion immediately.
3. Cardioversion	0.5-1 J/kg; if unsuccessful, increase to 2 J/kg. Confirm sync mode before every shock; sedate only if immediately available and it does not delay treatment.
4. Stable SVT?	Regular narrow-complex SVT: vagal maneuvers first; then adenosine by rapid push and immediate saline flush if access is ready.
5. Reassess	Treat cause, monitor continuously, obtain 12-lead only when it does not delay care, and notify destination.

Provider-level actions

Level	Approved actions
EMT	Oxygenation as indicated, vital signs/rhythm strip, identify compromise, and treat likely sinus-tachycardia causes within standing orders.
AEMT	All EMT care plus IV/IO and cause-directed supportive treatment. Do not delay transport or cardioversion for repeated access attempts.
Paramedic	All prior care plus synchronized cardioversion for compromise, vagal maneuvers/adenosine for stable regular narrow-complex SVT, and Medical Control for refractory stable SVT.

PC-05 Pediatric Narrow-Complex Tachycardia

SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not delay synchronized cardioversion for sedation.

• Do not give adenosine for an irregular, polymorphic, uncertain, or wide-complex rhythm.

• If pulseless at any point, begin CPR and use PC-04.

• Sinus tachycardia from fever, pain, dehydration, hypoxia, sepsis, toxin exposure, or anemia requires cause-directed treatment.

Medication reference

Medication	Approved dose / safeguard
Adenosine	0.1 mg/kg rapid IV/IO push with immediate saline flush; max 6 mg. Repeat 0.2 mg/kg; max 12 mg. Only for regular narrow-complex SVT.
Midazolam	0.1-0.2 mg/kg IV/IO/IN for cardioversion; max single 2 mg, max total 5 mg. Do not delay cardioversion; verify against pediatric medication reference.

Linked pathways

AR-05 Pediatric Airway; PC-04 Pulseless Arrest; PC-06 Wide-Complex Tachycardia; Medication Reference.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.